Bevacizumab and trifluridine-tipiracil (Lonsurf®)

Bevacizumab and trifluridine-tipiracil (Lonsurf®) is a cancer drug treatment. It is used to treat bowel cancer.

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What is bevacizumab and Lonsurf?

Bevacizumab and trifluridine-tipiracil (Lonsurf®) is a combination cancer treatment. It is used to treat bowel cancer.

Bevacizumab belongs to a group of targeted therapy drugs called monoclonal antibodies. It is also an angiogenesis inhibitor. Lonsurf is a chemotherapy drug made up of trifluridine and tipiracil.

It is best to read this information with our general information about cancer drug treatments and the type of cancer you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree (consent) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the electronic Medicines Compendium (eMC) website, which has patient information leaflets (PIL) for individual drugs.

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How bevacizumab and Lonsurf is given

You usually have bevacizumab in a chemotherapy day unit or clinic as an outpatient. You usually take Lonsurf as tablets at home.

During your course of treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- chemotherapy nurse or specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

Before or on the day of each treatment, you will have a blood test. This is to check that it is safe for you to have treatment.

You will meet with a doctor, nurse or pharmacist before you have treatment. They will talk to you about your blood results and ask how you have been feeling. If your blood results are okay, the pharmacy team will prepare your cancer drugs.

You have bevacizumab as a drip (infusion) into a vein. You have it through 1 of the following:

a cannula – a short, thin tube the nurse puts into a vein in the arm or hand

- a cannula – a short, thin tube the nurse puts into a vein in your arm or hand
- a central line – a fine tube that goes under the skin of your chest and into a vein close by
- a PICC line – a fine tube that is put into a vein in your arm and goes up into a vein in your chest
- an implantable port (portacath) – a disc that is put under the skin on your chest or arm and goes into a vein in your chest.

You have the first treatment of bevacizumab slowly over 90 minutes. If you do not have any problems, you have the next treatment over 60 minutes. After this, you usually have it over 30 minutes.

You take Lonsurf as tablets.

Your cancer team may also give you anti-sickness drugs and other medicines to take home. Take all your capsules or tablets exactly as they tell you to.

Your course of treatment

Your cancer team will discuss your treatment plan with you. They may give you a copy of the plan to take home. Follow any instructions your cancer team gives you.

Bevacizumab and Lonsurf is a 28-day (4-week) cycle.

You have an infusion of bevacizumab on day 1 of the cycle. You have this in the chemotherapy day unit or clinic.

You usually take Lonsurf tablets at home every day on days 1 to 5 of the cycle. You then have a break for 2 days on days 6 and 7. You take Lonsurf again on days 8 to 12.

On day 15 you have another infusion of bevacizumab at the chemotherapy day unit or clinic.

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You then have 13 days without any cancer treatment before starting the next cycle of bevacizumab and Lonsurf.

Taking Lonsurf tablets

Your cancer team will give you the tablets to take home. They may give you tablets of different strengths. Always take the tablets exactly as explained. This is important to make sure they work as well as possible for you.

The dose of Lonsurf may change each cycle. This may be because of side effects or changes to your weight. It is very important that you know how many tablets to take each time you start a new cycle.

Make sure you:

- take the tablets twice a day
- take them within 1 hour of eating your breakfast and evening meal
- swallow them whole with a glass of water
- do not chew, break or crush them.

If you forget to take the tablets, take the next dose at the usual time. Do not take a double dose to make up for the missed dose.

Other things to remember about your tablets:

- Wash your hands after taking them.
- Other people should avoid direct contact with them.
- Keep them in the original package and at room temperature, away from moisture, heat and direct sunlight.
- Keep them safe, where children cannot see or reach them.
- If you are sick (vomit) just after taking them, contact the hospital. Do not take an extra dose. Take your next dose at the usual time.
- Do not throw away unused tablets. Return them to your cancer team at the hospital.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

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It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Side effects while treatment is being given

Some people may have side effects while they are having this treatment or shortly after they have it.

Allergic reaction

Some people have an allergic reaction while having bevacizumab. The first infusion is the most likely to cause a reaction, so it is usually given more slowly than later infusions. Before treatment, you will have medicines to help prevent or reduce any reaction.

Signs of a reaction include:

- feeling hot or flushed
- shivering
- itching
- a skin rash
- feeling dizzy or sick
- a headache
- feeling breathless or wheezy
- swelling of the face or mouth
- pain in the back, tummy or chest.

Your nurse will check you for signs of a reaction during your infusion. If you feel unwell or have any of these signs, tell them straight away. If you do have a reaction, they can treat it quickly.

Sometimes a reaction happens a few hours after treatment. If you develop any of these signs or feel unwell after you get home, contact the hospital straight away on the 24-hour number.

Always call 999 if swelling happens suddenly or you are struggling to breathe.

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Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Risk of infection

This treatment can reduce the number of white blood cells in your blood. These cells fight infection. If your white blood cell count is low, you may be more likely to get an infection. A low white blood cell count is called neutropenia.

An infection can be very serious when the number of white blood cells is low. It is important to get any infection treated as soon as possible. If you have any of the following symptoms, contact the hospital straight away on the 24-hour number:

- a temperature above 37.5°C
- a temperature below 36°C
- you feel unwell, even with a normal temperature
- you have symptoms of an infection.

Symptoms of an infection include:

- feeling shivery and shaking
- a sore throat
- a cough
- breathlessness
- diarrhoea
- needing to pass urine (pee) often, or discomfort when you pass urine.

It is important to follow any specific advice your cancer treatment team gives you.

Your white blood cell count will usually return to normal before your next treatment. You will have a blood test before having more treatment. If your white blood cell count is low, your doctor may delay your treatment for a short time, until your cell count increases.

Bruising and bleeding

This treatment can reduce the number of platelets in your blood. Platelets are cells that help the blood to clot.

If the number of platelets is low, you may bruise or bleed easily. You may have:

- nosebleeds
- bleeding gums
- heavy periods
- blood in your urine (pee) or stools (poo)

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- tiny red, brown or purple spots that may look like a rash – these spots can be harder to see if you have black or brown skin.

If you have any unexplained bruising or bleeding, contact the hospital straight away on the 24-hour number. You may need a drip to give you extra platelets. This is called a platelet transfusion.

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Feeling sick

Your doctor, nurse or pharmacist may prescribe anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to. If you feel sick, take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids.

If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice and may change the anti-sickness drug to one that works better for you.

Diarrhoea

This treatment may cause diarrhoea. Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Sore mouth and throat

This treatment may cause a sore mouth and throat. You may also get mouth ulcers. This can make you more likely to get a mouth or throat infection. Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals.

Contact the hospital straight away on the 24-hour number, if:

-

a sore mouth or throat affects how much you can drink or eat

- your mouth, tongue, throat or lips have any blisters, ulcers or white patches.

They can give you advice, and mouthwash or medicines to help with the pain or to treat any infection. Follow their advice and make sure you:

- drink plenty of fluids
- avoid alcohol and tobacco
- avoid food or drinks that irritate your mouth and throat.

Loss of appetite

This treatment can affect your appetite. Don’t worry if you do not eat much for 1 or 2 days. But if your appetite does not come back after a few days, or if you are losing weight, tell your doctor, nurse or pharmacist. They can give you advice. They may give you food or drink supplements. Or they may suggest changes to your diet or eating habits to help.

Feeling tired

Feeling tired is a common side effect of this treatment. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

If you feel sleepy, do not drive or use machinery.

High blood pressure

This treatment can increase your blood pressure. This is called hypertension. A nurse will check your blood pressure regularly during your treatment. If you have headaches, nosebleeds or feel dizzy, let your doctor know. They can prescribe tablets to control high blood pressure.

Effects on the kidneys

This treatment can affect how the kidneys work. This is usually mild and goes back to normal after treatment ends. You will have blood tests and may have a urine sample test to check how well your kidneys are working

Drinking fluids helps protect your kidneys. The advice is usually to try to drink at least 2 litres (3½ pints) of fluid each day. But follow any advice from your doctor, nurse or pharmacist about how much is right for you.

Contact the hospital on the 24-hour number if you:

- have blood in your urine (pee)
- have foamy, frothy or bubbly urine
- are passing less urine or peeing less often than usual.

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Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Changes to your taste

Some foods may taste different or have no taste. Try different foods to find out what tastes best to you. You may also get a bitter or metallic taste in your mouth. Your doctor, nurse or pharmacist can give you advice. It might help to try:

- sucking sugar-free sour or boiled sweets
- eating cold foods
- eating sharp-tasting fresh fruit.

We have more information about coping with changes to taste.

Tummy pain

You may get pain in your tummy (abdomen), or have indigestion. Your doctor, nurse or pharmacist can give you advice or treatment to help. Contact the hospital straight away on the 24-hour number if your symptoms do not go away, or get worse.

Bevacizumab may cause a hole (perforation) in the bowel. This is more likely if you have also had radiotherapy to the pelvis (lower part of the tummy).

Contact the hospital on the 24-hour number straight away if the tummy pain is severe or is not getting better.

It is also very important to let them know if:

- you have bleeding from the back passage (rectum) or black stools
- you are vomiting up blood
- your vomit looks like coffee grounds.

Headaches

This treatment may cause headaches. If you have headaches, tell your doctor, nurse or pharmacist. They can give you advice about painkillers that may help. Tell them if the headache does not get better, or gets worse.

Eye problems

Your eyes may become watery. Your doctor can prescribe eye drops to help with this. Always tell your doctor, nurse or pharmacist if you notice any changes in your vision.

Effects on the lungs

This treatment can cause changes to the lungs. Tell your doctor, nurse or pharmacist if you develop:

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- a cough that does not go away
- wheezing
- breathlessness.

You should also tell them if any existing breathing problems get worse. You may have tests to check your lungs.

Constipation

This treatment can cause constipation. Constipation means that you are not able to pass stools (poo) as often as you normally do. It can become difficult or painful. Here are some tips that may help:

- Drink at least 2 litres (3½ pints) of fluids each day.
- Eat high-fibre foods, such as fruit, vegetables and wholemeal bread.
- Do regular gentle exercise, like going for short walks.

If you have constipation, contact the hospital on the 24-hour number for advice. They can give you drugs called laxatives to help.

If you have not been able to pass stools for over 2 days and are being sick, contact the 24-hour number straight away.

Hair loss

Your hair may get thinner but you are unlikely to lose all the hair from your head. Hair loss is almost always temporary and your hair will usually grow back after treatment ends. Your nurse can talk to you about ways to cope with hair loss.

Skin changes

Cancer treatment can affect your skin. If your skin feels dry, try using soap-free cleansers and unperfumed moisturising cream every day.

This treatment can also:

- cause a rash, which may be itchy
- make your skin darker in some areas
- make you more sensitive to the sun.

If you have had radiotherapy, the treated area may become sore. If you have white skin, the area may become red. If you have black or brown skin, the area may become darker.

Your skin may burn more easily during treatment and for several months after. Use a suncream of at least SPF 30. SPF stands for sun protection factor. Cover up with clothing and wear a hat.

Always tell your doctor, nurse or pharmacist about any skin changes. They can give you advice or prescribe creams or medicines to help.

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Muscle or joint pain

You may get pain in your muscles or joints with this treatment. If this happens, tell your doctor, nurse or pharmacist. They can give you painkillers and advice. They can also tell you if any of the painkillers you usually take are suitable.

Effects on the liver

This treatment can affect how your liver works. This is usually mild.

You will have blood tests to see how well your liver is working.

Numb or tingling hands or feet (peripheral neuropathy)

This treatment may affect the nerves in your fingers and toes. This can cause numbness, tingling or pain in your hands or feet. This is called peripheral neuropathy. You might find it hard to do fiddly tasks such as fastening buttons or tying shoelaces.

If you have these symptoms, always tell your doctor, nurse or pharmacist. They sometimes need to change the dose of the drug.

Hand-foot (palmar-plantar) syndrome

This treatment can affect the palms of your hands and the soles of your feet. This is called palmar-plantar or hand-foot syndrome.

If you have white skin these areas may become red. If you have black or brown skin, these areas might get darker.

The skin on the palms of your hands and the soles of your feet may:

- be sore
- be painful, tingle, or swell
- peel, crack or blister.

If you have any of these symptoms, contact the hospital straight away on the 24-hour number. They can give you advice. This is especially important if you have any broken skin or if walking is difficult. They can prescribe creams and painkillers to help.

You can care for your hands and feet by:

- keeping your hands and feet cool by washing in cool water
- gently moisturising your hands and feet regularly
- wearing gloves to protect your hands and nails when working in the house or garden
- wearing loose cotton socks and avoiding tight-fitting shoes and gloves.

Changes in the way your heart works

This treatment may affect the heart. Contact the hospital on the 24-hour number if:

- you feel that your heart is beating too fast
- you have ankle swelling.

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These could be signs that bevacizumab is affecting your heart.

Always call 999 if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Slow wound healing

Wounds may take longer to heal while you are being treated with bevacizumab. If you have any wounds that are not healing or look infected, speak to your doctor or nurse straight away.

If you need to have surgery or invasive dental work, you will stop having bevacizumab about 4 to 6 weeks before the operation. You can start it again when the wound is fully healed.

Blockage in the bowel

This treatment can sometimes cause your bowel to become blocked (bowel obstruction). Contact the hospital straight away on the 24-hour number if you stop passing poo and have severe pain in your tummy.

Fistula

Rarely, bevacizumab can cause a fistula. A fistula is an abnormal opening that connects 2 or more parts of the body. If this happens, it is more likely to be in the part of the body affected by the cancer.

Jaw problems (osteonecrosis)

Rarely, bevacizumab may cause osteonecrosis of the jaw. This is when bone tissue in the jaw becomes damaged and dies. Symptoms of osteonecrosis include:

- pain
- swelling
- redness of the gums
- loose teeth.

Tell your cancer doctor and dentist straight away if you have any of these symptoms.

Fluid build-up (oedema)

Sometimes fluid builds up in the legs and ankles. This is called oedema. Fluid may also build up in the linings of the lungs. This is called a pleural effusion.

Oedema and pleural effusion may be treated with drugs that help you to you pass more urine (pee). These drugs are called diuretics. Always tell your doctor if you notice any swelling. It is important to contact your doctor or the hospital straight away if you:

- develop a cough
- have chest pain
- feel more breathless than usual
- gain weight suddenly.

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Voice changes

You may notice some voice changes or hoarseness. Talk to your doctor, nurse or pharmacist if you are worried about this.

Blood clot risk

Cancer and some cancer treatments can increase the risk of a blood clot. Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Effects on the brain

Rarely, this treatment causes a brain condition that can be serious. You can make a full recovery from this. But it must be diagnosed and treated quickly.

This condition can cause:

- a headache that does not get better
- drowsiness or confusion
- changes in eyesight
- fits (seizures).

If you have any of these symptoms, it is important to either:

- contact the hospital straight away on the 24-hour number
- go to the hospital straight away.

You should not drive yourself to hospital.

Other information

Lactose

This treatment may contain lactose. If you have a lactose allergy or intolerance, ask your doctor, nurse or pharmacist for more information.

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Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- herbal drugs and complementary or homeopathic therapies
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and coronavirus (covid). They may also recommend other vaccines, such as Shingrix® for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Contraception

Your doctor, nurse or pharmacist will advise you not to get pregnant or make someone pregnant while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to breastfeed while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can get pregnant or make someone pregnant. If you are worried about this, it is important to talk with your doctor before you start treatment.

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Sex

It is possible that small amounts of chemotherapy may be passed on through vaginal fluids or semen. If you have sex during this treatment, your cancer team will usually advise using condoms or a dental dam to protect your partner.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References

Visit the electronic Medicines Compendium (eMC) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

Cancer Information Development team

Author

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Our cancer information meets the PIF TICK quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about how we produce our information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

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We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as ‘you’ so that everyone feels included. Where clinically necessary we use the terms ‘men’ and ‘women’ or ‘male’ and ‘female’. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information here.

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.

Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.

Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don’t want to call.

Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.

Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

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Get help

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